8.3 Diabetic Ketoacidosis*

Assessment	Nursing Diagnosis	Plan / Expected Outcome	Nursing Interventions	Evaluation
Subjective • "I am very thirsty and have been vomiting." **Objective** • Glucose 480 mg/dL • Ketones positive; pH 7.15 • Kussmaul breathing • Dry mouth; HR 120/min	**ACTUAL** Inadequate fluid volume related to osmotic diuresis and vomiting as evidenced by dry mucosa, tachycardia and hyperglycemia.	• Vital signs stable and urine output adequate within 6–8 hours.	• Give IV fluids according to the DKA protocol. • Measure urine hourly and keep an intake and output chart. • Check vital signs and consciousness hourly. • Check glucose hourly and electrolytes as ordered.	• Achieved once he is rehydrated and stable.
Subjective • "I feel I cannot catch my breath." **Objective** • Kussmaul breathing • pH 7.15, HCO_3^- 10 mmol/L • Fruity breath	**ACTUAL** Ineffective breathing pattern related to respiratory compensation for metabolic acidosis as evidenced by Kussmaul respirations and low pH.	• Normal breathing pattern within 24 hours as the acidosis clears.	• Follow respiratory rate, depth and blood pH as ordered. • Run the insulin infusion and fluids per protocol. • Check potassium before and during insulin; replace as prescribed. • Watch the ECG for signs of potassium disturbance.	• Outcome reached when breathing and pH normalize.
Subjective *No specific complaint* **Objective** • Insulin infusion	**RISK** Risk for physical injury as evidenced by risk factors: hypoglycemia associated with intravenous insulin infusion.	• Glucose falls gradually without hypoglycemia.	• Check glucose hourly. • Add dextrose to the fluids when glucose reaches the protocol threshold. • Continue the insulin infusion until ketosis clears and subcutaneous insulin has been given.	• Considered met if hypoglycemia does not occur.

* **References for this care plan:** [3], [35], [127], [128]. Full entries are listed in the References section of the CPMN book.